

UTI in men

(PHE 2020; NICE 2018, NG112; NICE 2018, NG109;
EAU guidelines on urological infections, 2024)



Red Whale

GEMS
Guidelines & Evidence Made Simple

Symptoms and signs suggest UTI

Do **not** use urine dipstick to rule out infection in males (unreliable)

Consider other causes
e.g. sexually transmitted
infection

<65y

Age

≥65y

Febrile?

Yes

Consider pyelonephritis,
prostatitis or sepsis
(consider validated sepsis score)
ADMIT if acutely unwell

No

Delirium?

Yes

Delirium causes (PINCH ME)

- Pain
- other Infection
- poor Nutrition
- Constipation
- poor Hydration
- other Medication
- Environment change

Screen and treat as appropriate

No

Yes

Catheter

No

Check for blockage. Consider catheter
removal or replacement.

Remember: smelly or cloudy urine does
not reliably indicate infection, and
asymptomatic bacteriuria does not need
treatment in a catheterised male.

Decision made to treat as UTI

Self-care

- Drink enough fluid to avoid dehydration; consider paracetamol +/- ibuprofen
- PHE leaflet for older adults and carers (RCGP UTI toolkit – link in main article)
- *No evidence* for cranberry or urine-alkalinizing agents

Antibiotics: see below

Does your patient know how to collect an MSU?

Consider patient information leaflet or online
resource (see main article)

Obtain sample for culture
BEFORE starting treatment

- Review MSU result
- Amend antibiotics as required
- Imaging or referral indicated?
- Routine follow-up MSU not needed

Consider imaging/referral if:

- Could this be cancer? Refer on suspected cancer pathway.
- Suspected upper UTI (may need urgent assessment)
- Treatment failure
- Persistent haematuria (visible or non-visible) (may need suspected cancer pathway referral)
- Recurrent UTI (2 infections in 6 months or 3 in 12 months)
- Voiding dysfunction without obvious cause, e.g. BPH
- Underlying cause, e.g. stone, obstruction, previous GU surgery, previous pyelonephritis

Which antibiotic? (NICE NG109, 2018)

First line	Trimethoprim 200mg bd for 7d or Nitrofurantoin 100mg MR bd for 7d (if eGFR≥45ml/min)
Second line (no improvement after 48h of 1 st line, or when first choice not suitable)	If symptoms of pyelonephritis see NICE pyelonephritis (NICE 2018, NG111) Consider prostatitis and follow NICE acute prostatitis (NICE 2018, NG110) NICE do not directly suggest a second line antibiotic choice Discuss with local microbiology team for further advice, if required

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